جوستاف روسي انترناشيونال - مصر (EPE) *

# طلب علاج أول مرة / متردد

**إسم المريض رباعياً :** نيفين جاد سعيد ناروز
**المحافظة :** القاهره **الحي / المدينة :** عابدين **العنوان :** القاهره
**رقم البطاقة :** 27603050100101 **رقم التليفون :** 01287957895 **تاريخ تسجيل الإستمارة :** 11-10-2026
**العمر(تاريخ الميلاد) :** 05-03-1976 **المهنة :** ربه منزل

## الإقرار

**أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك**

**الإخصائي الاجتماعي** **المقر بما فيه**
**التوقيع:**(----------) **التوقيع:**(----------)

**التشخيص الإكلينيكي :** سرطان بالثدي (مبادرة)
**نتائج الأبحاث :** (مرفق)
**التشخيص النهائي :** سرطان بالثدي (مبادرة)
**خطة العلاج :** أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
**التعليق على التشخيص :** ----------

## تقرير اللجنة الثلاثية المتخصصة

**الاسم:** د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

**تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.**

## أعضاء اللجنة

**ا.د/** ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

## قرار اللجنة

**التشخيص النهائي :** ----------
**خطة العلاج :** ----------
**المستشفي :** ----------**قسم :** ----------
**جهة العلاج :** ----------**نفقات :** ----------
**ملاحظات :** ----------

| رئيس | مدير عام | ختم |
|---|---|---|
| المجلس الطبي المتخصص | مديرية الشئون الصحية | شعار الجمهورية |

Name: نيفين جاد سعيد ناروز
National ID: 27603050100101

GUSTAVE ROUSSY INTERNATIONAL

# تقرير طبي

السيد

تحية طيبة وبعد,,

| | |
|---|---|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم: | رقم طبي: |

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

| | |
|---|---|
| تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd | تاريخ وساعة الخروج : PM 01:17 05-07-2026 |

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

| | |
|---|---|
| التاريخ : 05-07-2026 | الوقت : -- --:-- |
| | الطبيب المعالج : د/شيماء لاشين |

النقابة العامة للمحامين
مشروع الرعاية الصحية
للمحامين وأسرهم

# شهـــادة

تشهد النقابة العامة للمحامين بان الأستاذ/نيفين جاد سعيد ناروز

المقيدة برقم (٢٦٤٣٠٠) غير مشتركه بمشروع العلاج بالنقابة العامة لسنة ٢٠٢٥

وهذه شهادة منا بذلك لتقديمها الى من يهمه الأمر دون ادنى مسئولية على النقابة.

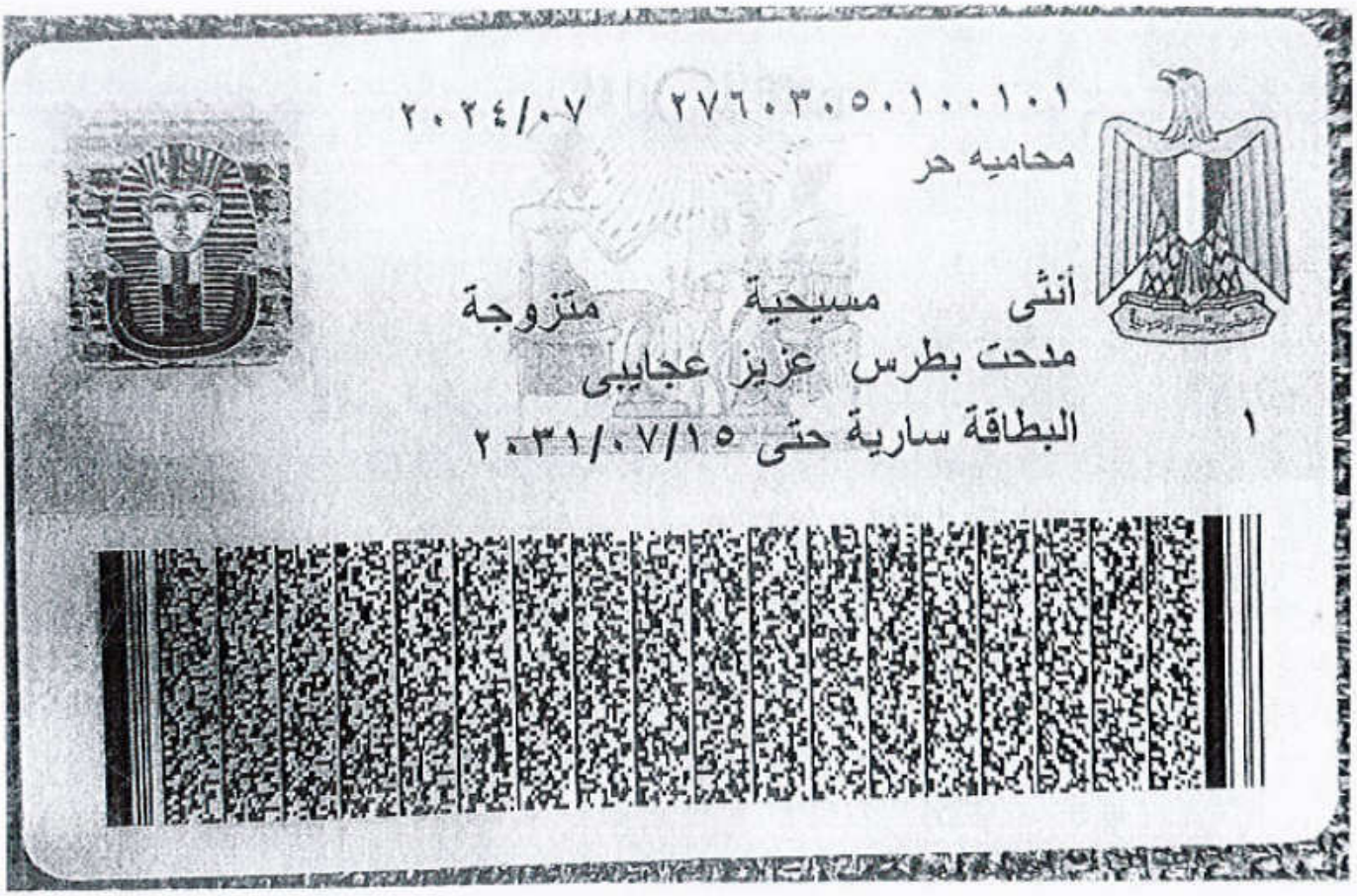
وتم سداد الرسم بموجب ايصال رقم( ١٠٠٠٣٦٧٥٦ )

بتاريخ ٢٠٢٥/١١/١٧ رقم قومى(٢٧٦٠٣٠٥٠١٠٠١٠١)

مدير إدارة بطاقات العلاج

التوقيع/ ١٧/١١/٢٠٢٥

المختبر
البرج سكان
معامل الفا
16191
معامل حسّاب
Hassab Labs
Dar El Oyoun







National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

| | |
|---|---|
| Name: نيفين جاد سعيد | |
| MR. : 347 | |

Detailed Medical Report

السيد

تحية طيبة وبعد,,

| | | | |
|---|---|---|---|
| إيماء للطلب المقدم من | | بشأن تقرير طبي للمريض | |
| بقسم: | | رقم طبي: | |

نشرف بأن نوضح فيما يلي تقريرا عن حالته

| | | | |
|---|---|---|---|
| تاريخ وساعة الدخول : | | تاريخ وساعة الخروج : | |

وبعد الكشف والفحوصات والمتابعة وجد انه

**49 y sa1.52 ef 63 % 4/12 prem left breast cancer ct4b n1 m0 er8/8 pr 7/8 her 3+ rec 5 cycles taxotere carboplatin on exam left uoq mass2x2 left ax ln 2x1 for c6 taxotere 120 mg carboplatin auc 5 c6 phesgo for mdt for surgery**

**FOR CYCLE 8 PHESGO >> WAITING PATHOLOGO OF SURGERY**

**for c9 phesgo + (femara) depending on القرار cbc pending zoladex after E2 levels zometa after dental care pathology report : ypT0 , ypN0 , free margins , no lymphovascular invasion ,with Complete response to NACT**

**for c10 phesgo echo 10/3 ef 61% FOR FEMARA + SUPPORTIVE**

**pt did left ssm sln and reconstruction cr ypt0 ypn0/6 7/3/2026 for zoladex femara**

**for femara + supportive ttt +c12 phesgo for E2 levels next cycle**

**Patient is Due this cycle For new ECHO Patient was instructed to do a new ECHO before next week**

**9/8 ECHO 63% ef FOR C13 PHESGO + FEMARA + SUPP ZOLADEX 16/8 PHARMACY**

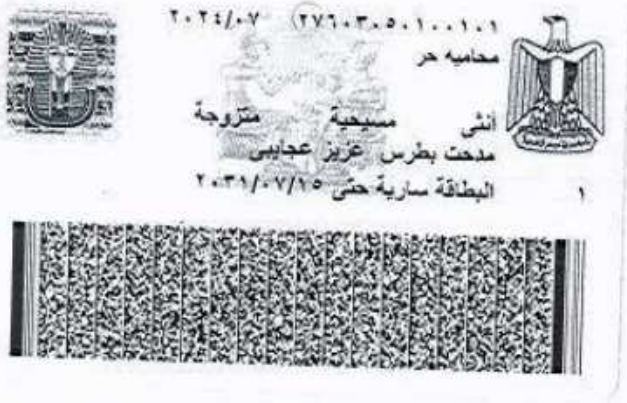
تحريرا في ,

| | | | |
|---|---|---|---|
| التاريخ : | 19-09-2026 | الوقت : | |

الطبيب المعالج : dina makram

يعتمد ,




National ID - Front & Back
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جوستاف روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

03/28/2026

# MDT FORM

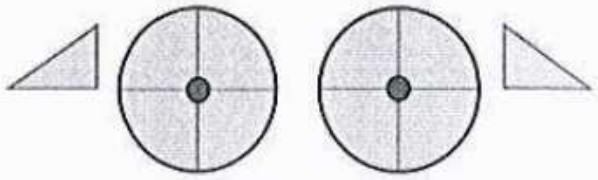
| | | | |
|---|---|---|---|
| **Name:** | Niveen Jad Saeed . | **MRN:** | 347 |
| **Age:** | 50 Y | **ID:** | 27603050100101 |
| **Menopausal Sta** | AMMENORRHEA POST NACT | **Phone:** | 01201109076 |
| **NRS SCORE:** | | **Marital Status:** | MARRIED |
| **BMI:** | | **Family History:** | -VE |
| **CO Morbidity:** | M:FREE | **Reason Of MDT:** | POST OP |

## Clinical Data:

**BX:** LEFT IDC

**ER:** 8/8 **PR:** 7/8 **HER2:** +3 **KI67:** 25

R L



**TNM**

**Sonomammography:**

**Mets Workup:**

**History:**

LEFT IDC RECIEVED NACT UNDERWENT LEFT SM+SLNB +EXPANDER INSERTION
Diagnosis:
Left breast cancer (Post-NACT), Left simple mastectomy & Sentinel lymph nodes biopsy:
- No residual viable remnants of the previously diagnosed invasive tumor.
- Ductal carcinoma in situ: Absent.
- Nipple & areola: Free.
- Deep margin: Free.
- Sentinel lymph nodes: Six lymph nodes with no viable metastatic deposits, LNs (0/6)
(Multiple lymph nodes showing effect of therapy).
- Histologic response to therapy: Pathological complete response to therapy.
STAGE:ypT0,ypN0(SLN)

**Recommendations:**

FOR POSSIBILTY OF ADJUVANT TTT

**MDT Outcome & Plan**

FOR ADJ PHESGO up yo 17 cycles
PORT
ADJ femara consider zoladex after E2 LEVEL if still pre menopausal
zometa every 6 ms

## SIGNATURE

| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY | Nutrition | Psychiatric |
|---|---|---|---|---|---|
| | | | | | |

[redacted]

MDT Form
[[DOCTYPE:MDT_FORM]]

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| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / نيفين جاد سعيد تاورز | Request No. | : 13005329 |
| Patient No. | : 27603050100101 | Request Date | : 07/03/2026 |
| Path No. | : 631/26 | Verification Date | : 12/03/2026 |
| Sex | : Female | Financial Category | : جوستاف روسي - نفقة الدولة |
| Age | : 50 Y | Department | : Out Patient |
| Referreing Doc | : | Registered by | : .marwa mohamed youss |

## Pathology Report

Clinical Data: Left breast cancer, Post-NACT.
Nature of Specimen: Left simple mastectomy & Sentinel lymph nodes biopsy.

Gross Examination:
* Left simple mastectomy specimen measured 22x16x5 cm, with covering skin ellipse measured 22x7 cm. Nipple & areola looked unremarkable grossly.
* Serialling showed a tumor bed site 11x7x3 cm, located 0.5 cm from the least deep margin.
* Sentinel lymph nodes: Fibrofatty tissue piece measured 5x5x1 cm, entangling 6 lymph nodes (Only 5 submitted for frozen), the largest measured 3x2x0.5 cm.
* 17 Blocks:
1:13- T.B.S., 14- N&A., 15- Deep margin., 16&17- SLNs..
- Gross was done by; Dr. Heba Hassan.

** Frozen (intra-operative) results:
- Five sentinel lymph nodes free of viable metastatic deposits (Six lymph nodes are detected but only five submitted for frozen).
** Frozen was examined by Prof. Dr. Gamal abd-El-Aty.

Microscopic Examination:
* Histological type: No residual viable remnants of the previously diagnosed invasive tumor.
- Cellularity (0%).
* Angioinvasion: Absent.
* Ductal carcinoma in situ: Absent.
* Additional findings: Effect of therapy with microcalcifications.
* Nipple & areola: Free.
* Deep margin: Free.
* Sentinel lymph nodes: Six lymph nodes free of viable metastatic deposits (0/6) (Multiple lymph node showing effect of therapy).

Verified by :
Dr.Duaa Mohamed



CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / نيفين جاد سعيد تاورز | Request No. | : 13005329 |
| Patient No. | : 27603050100101 | Request Date | : 07/03/2026 |
| Path No. | : 631/26 | Verification Date | : 12/03/2026 |
| Sex | : Female | Financial Category | : جوستاف روسي - نفقة الدولة |
| Age | : 50 Y | Department | : Out Patient |
| Referreing Doc | : | Registered by | : .marwa mohamed youss |

## Pathology Report

* Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:
Left breast cancer (Post-NACT), Left simple mastectomy & Sentinel lymph nodes biopsy:
- No residual viable remnants of the previously diagnosed invasive tumor.
- Ductal carcinoma in situ: Absent.
- Nipple & areola: Free.
- Deep margin: Free.
- Sentinel lymph nodes: Six lymph nodes with no viable metastatic deposits, LNs (0/6) (Multiple lymph nodes showing effect of therapy).
- Histologic response to therapy: Pathological complete response to therapy.
- Stage: ypT0, ypN0 (sln).

Signature
Dr. Walaa Ibrahim Ghoneim.

Verified by :
Dr.Duaa Mohamed



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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

USTAVE / OUSSY
ERNATIONAL

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / نيفين جاد سعيد تاورز | Request No. | : 13004506 |
| Patient No. | : 27603050100101 | Request Date | : 25/08/2025 |
| Path No. | : 2625/25 | Verification Date | : 28/08/2025 |
| Sex | : Female | Financial Category | : جوستاف روسي - نفقة الدولة |
| Age | : 49 Y | Department | : Out Patient |
| Referreing Doc | : | Registered by | : .marwa mohamed youss |

## Pathology Report

Clinical Data: Left breast 12 O'clock lesion (BIRADS 4a) & 1 O'clock lesion (BIRADS 4c).
Nature of Specimen:
1- Left breast 12 O'clock lesion, Tru-cut biopsy, collected on 24/08/2025.
2- Left breast 1 O'clock lesion, Tru-cut biopsy, collected on 24/08/2025.

Gross Examination:
Two specimens were received as follow:
1- Left breast 12 O'clock lesion: Two grey white tissue cores, measured 1 cm & 0.7 cm long. Totally submitted.
2- Left breast 1 O'clock lesion: Four grey white tissue cores, the longest was 0.7 cm. Totally submitted.
Technique:
- Sections were prepared from the paraffin block (2), processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:
1- Left breast 12 O'clock lesion:
Histopathological examination revealed cores of breast tissue showing diffuse stromal fibrosis and focal adenosis. Some of the proliferating ducts are irregularly dilated. The ducts are lined by inner columnar and outer myoepithelial cells.

2- Left breast 1 O'clock lesion:
* Histological type: Invasive duct carcinoma.
* Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
* 5% Intermediate grade DCIS (Solid pattern).
* Angioinvasion: Absent.

Verified by :
Dr.Duaa Mohamed



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Pathology Report
[[DOCTYPE.PATHOLOGY_REPORT]]

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / نيفين جاد سعيد تاورز | Request No. | : 13004506 |
| Patient No. | : 27603050100101 | Request Date | : 25/08/2025 |
| Path No. | : 2625/25 | Verification Date | : 28/08/2025 |
| Sex | : Female | Financial Category | : جوستاف روسي - نقلة الدولة |
| Age | : 49 Y | Department | : Out Patient |
| Referreing Doc : | | Registered by | : .marwa mohamed youss |

## Pathology Report

* Perineural invasion: Absent.
* IPT (Positive control showed appropriate reactivity) (On block-2):
- ER: Strong nuclear reaction in 70% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 50% of tumor cells (score 7/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 25%.

Diagnosis:
1- Left breat 12 O'clock lesion, Tru-cut biopsy:
- Fibroadenosis.
2- Left breast 1 O'clock lesion, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 2, with 5% Intermediate grade DCIS (Solid pattern).
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Positive (score 3+).
- Ki67%: 25%.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane

Verified by :
Dr.Duaa Mohamed



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

# OP Notes

Patient: **15467218 : Neven Gad Said** Female, EGYPTIAN, 49 Year(s), Tel: 93095818

Department: BREAST SURGERY Doctor : AISHA MASOUD SLOUM AL SHUKAIRI Consultant: AISHA MASOUD SLOUM AL SHUKAIRI

## Vital Signs

## Examination Notes

**23/07/2025 11:07 Aisha Masoud Sloum Al Shukairi - Breast Surgery**

Today discussed in MDT , need further workup
CT staging and breast MRI.
Patient called , she is in Salalah . she will come back on 29/7
she is infomred about the need for MRI breast
OPD on 5/8 to disclose the diagnosis and CT scan ( booked as walk in the same day)_.

## Diagnosis

**Final:** Occult breast cancer

## Procedures

| Date | Procedure | Done Date | Report |
|---|---|---|---|
| 23/07/25 | CHEST+ABDOMEN+PELVIS CT | 05/08/25 11:30:08 | Report Date : 05/08/25 11:46:12 |

**CT Abdomen and pelvis with IV contrast (Portovenous)**

**Indication:** left breast occult cancer for staging.
**Comparison:** None.
**Findings:**

The liver is normal in size and shape. No focal mass lesions seen. Portal vein is patent. There is mild intrahepatic biliary dilatation.

The gallbladder showing 3 cm gallstone with no acute complications.

Spleen, pancreas and both adrenals are unremarkable.

Both kidneys are normal in size, shape and position. No renal or ureteric calculi. No hydronephrosis or hydroureter. Left upper pole Bosniak 1 simple renal cortical cyst measuring 5.3 x 5 cm size.

The stomach is distended and appears unremarkable. The small bowel and large bowel are unremarkable.

Urinary bladder is grossly unremarkable.

No free fluid seen.No evidence of pneumoperitoneum.

IUCD seen in the endometrial cavity, unremarkable ovaries, no pelvic mass.

No enlarged abdominal lymph nodes are seen.



[[DOCTYPE:CT_SCAN]]

*ID: 4827759* *Name: Nevein Gad Saeed Naroz*
*Date: 20/09/2025* *Referred by: Prof. Mohamed Alkilani*

# Echocardiography Report

| Parameter | Value | Normal range for age |
|---|---|---|
| AO | 2.9 | 2.0 - 4.0 cm |
| LA | 3.4 | 2.0 - 4.0 cm |
| LVED | 3.8 | 3.5 - 5.6 cm. |
| LVES | 2.3 | - 3.5 cm |
| IVS(D) | 0.8 | 0.6 - 1.1 cm |
| LVPW(D) | 0.8 | 0.6 - 1.1 cm |
| FS% | 41 % | 25 - 45 % |
| EF% | 73 % | > 55% |
| RV(D) | 2.0 | 0.7 - 2.5 cm |

**Measurements (M-mode):**

## *Comment:*

- *Normal LV internal dimensions and normal global contractility.*
- *No evidence of regional wall motion abnormalities at rest.*

*Grade I, pattern of impaired LV diastolic function.*
*Mildly thickened mitral valve (0.65 cm), with good excursion.*
*Mild mitral incompetence. Regurgitant jet area = 2.1 cm2.*

- *Normal aortic valve and normal flow pattern.*

*Mild tricuspid incompetence. PPASP = 29 mmHg.*
*No evidence of significant pulmonary hypertension or RV pressure overload pattern.*

- *No detectable intra cardiac masses or thrombi.*
- *Normal pericardium and no detectable pericardial effusion.*

*Best regards,*

*Mohamed Shalaby*
*MD Cardiology*
*Membership ASE*



Echo
[[DOCTYPE ECHO]]

*Name: Nevein Gad Saeed Naroz* *ID: 4827759*

*Date: 18/09/2025*

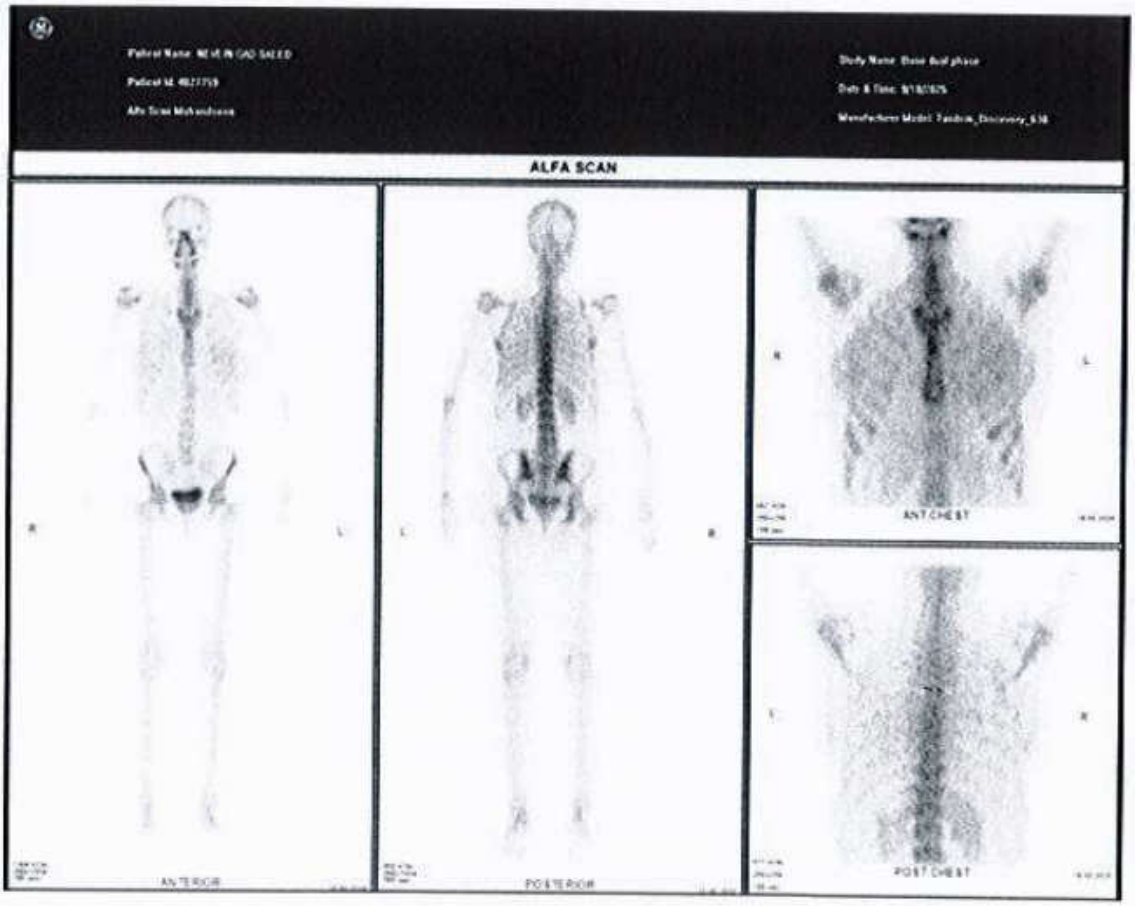
# $^{99m}$Tc-MDP Whole body Bone Scan

**Clinical data:**

| | |
|---|---|
| Indication | Left breast cancer. |
| Tumor markers | Not available. |
| Skeletal trauma/ surgeries | No |
| Previous imaging | Not available |
| Previous bone scan | No |

**Technique:**

| | |
|---|---|
| Injected dose | 740 MBq |
| Images | Delayed whole body + spot images |




Bone Scan
[[DOCTYPE:BONE_SCAN]]

**Findings:**

- No abnormal tracer uptake is seen over the surveyed skeleton that would account for bone metastases.
- Mild degenerative changes of the spine are noted.

**Impression:**

- **No scintigraphic evidence of bone metastases.**

*Dr. Sherif El-Refaei, MD, EBNM* *Dr. Hagar Nageh*

*4827759*

Bone Scan
[[DOCTYPE:BONE_SCAN]]

# OP Notes

Patient: **15467218 : Neven Gad Said** Female, EGYPTIAN, 49 Year(s), Tel: 93095818

Department: BREAST SURGERY Doctor : AISHA MASOUD SLOUM AL SHUKAIRI Consultant: AISHA MASOUD SLOUM AL SHUKAIRI

## Vital Signs

## Examination Notes

**06/08/2025 12:53 Aisha Masoud Sloum Al Shukairi - Breast Surgery**

Telemedicine appointment to trace MDT plan:

---

MDT Breast:
Case of left breast cancer with axillary LN mets, triple positive
Imaging reviewed: 1 small lesion in the breast, the bigger lesion is benign (hamartoma), pathologic axillary LN
Staging: lung nodules for follow up

---

Plan:
1. For NAC DCHP x6
2. Bone scan
3. ECHO
4. Surgical plan: BCS + axillary staging. (clip inserted)
5. Genetic testing

---

patient called, she is not working ( her husband is a teacher ) , she is welling to buy the medication from Egypt and get the injections and follow up in Royal Hopistal
referral to oncology given for appointment .

## Diagnosis

**Final:** brease cancer

## Referrals

**Internal Referal**

Referred to Medical Oncology On: 06-08-2025 13:00
Requested By Aisha Masoud Sloum Al Shukairi(Breast Surgery)
Notes: newly diagnosed breast cancer , discussed in MDT for neuadjuvent systemic therapy kindly give appointment

Replied by Hind Mohammed Ibrahim Ahmed
Unit (MEDICAL ONCOLOGY) On: 06-08-2025 13:06
Opinion: Thank for referral:as per Hospital role,expatriate patient to be managed at private sector Dr.Sahar will contact the patient to explain the plan

## Appointments

Printed on 11-Aug-2025 10:47 by (86012) Sahar Mohamed Awad



MDT Form 2025-08-11
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# Procedure Visit

Patient: **15467218 : Neven Gad Said** Female, EGYPTIAN, 49 Year(s), Tel: 93095818

Department: RADIOLOGY

Doctor : AISHA MASOUD SLOUM AL SHUKAIRI

Consultant: SAQAR HAMED ISSA AL TAII

## Vital Signs

## Lab Investigations

| Execute Date | Test Name | Released Date |
|---|---|---|
| 03/08/25 11:13 | Histopathology Examination | 05/08/25 11:47 |

GROSS :-
Recieved are multiple linear cores of grey soft tissue measuring in aggregate 1.5 x 0.6 cm. All are submitted in one block.
SYNOPTIC REPORT :-
Left breast lesion, needle core biopsy:
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Tumor Histologic type: Invasive duct carcinoma, NST. Category B5b.
Tumor Grade:
- Tubular score: 2.
- Nuclear pleomorphism score: 3.
- Mitotic rate count: 9/10HPF, score: 2.
- Overall provisional grade: 2.

DCIS garde, patterns & necrosis: high grade, cribriform with focal necrosis.
Microcalcifications: Present, focal.
Biomarker studies:
-Oestrogen receptors: Positive, 80% of tumour cells show moderate nuclear staining (Allred score 5+2 =7/8).
-Progestrone receptors: Positive, 60% of tumour cells show moderate nuclear staining (Allred score 4+2 = 6/8).
-HER-2 immunostain score:
-Positive, score 3+ (complete, intense circumferential membrane staining in more than 10% of invasive tumour cells).
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Snomed Classification :
T-04010 - BREAST FEMALE. NOS.
M-85003 - Carcinoma-invasive duct
US - URGENT SMALL BIOPSY
Z - . . COPY OF THE REPORT SENT TO - "THE CANCER REGISTRY", THE ROYAL HOSPITAL.
P24 - 24 workload points
Reporting Doctors:-
DR.ZAKIYA IBRAHIM HAMED AL AJMI
Consultant
Department of Histopathology

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| | |
|---|---|
| PATIENT NAME | NEVIEN SAAD SAED |
| DATE | 01 /02 /2026 |

## BREAST HIGH RESOLUTION ULTRASONOGRAPHY REVEALED:

Indication: Follow up after neoadjuvant chemotherapy of pathologically proved Left breast carcinoma.

- Homogenous glandular background parenchyma.

Left breast:

- Complete disappearance of the previously described malignant mass seen at 2 o'clock position of the left breast .
- The previously described well circumscribed lesion seen in the left breast at 11-12 o'clock position ,zone 1 ,now measures ( 6 x 2 cm )...mostly fibroadenolipoma.
- No other suspicious masses in other quadrants.
- Multiple cysts are seen scattered in the left breast parenchyma,the largest seen at 1 o'clock and measures (11 x13 mm ).
- Normal skin, subcutaneous fat and nipple areola complex on both sides.
- Restoration of the shape and hila of the previously described left suspicious axillary node.

Right breast :

- Homogenous glandular background parenchyma.
- Multiple subcentimteric cysts are seen scattered .
- The previously described well circumscribed lesion seen at 12 o'clock measures ( 8 x 6 mm ).
- Normal skin, subcutaneous fat and nipple areola complex.
- Non specific right axillary LNs.

Sonography
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